



## 门诊病历

科室：急诊科

姓名：苏厚成

性别：男

门诊号：24152016

联系方式：15982372342

年龄：26 岁

婚姻状况：其他

### 2026-02-27 16:34 初诊记录

主 诉：车祸伤后胸背部疼痛伴上腹部不适 2 小时

现 病 史：无特殊

既往史：无特殊

过敏史：无

体格检查：1.T8 椎体压缩骨折，被压缩约 1/4 椎体高度，对应层面椎管无明显狭窄。

2. 肝脏、胆囊、胰腺、脾脏、双肾及扫及肠管未见明显创伤性改变。

检验检查：上腹部 + 下腹部，胸椎 CT 平扫

初步诊断：1. 胸椎 T8 椎体压缩性骨折 2. 腹部闭合性损伤待排（CT 未见明确实质脏器损伤）

处 理：1. 急诊留观，持续监测生命体征，嘱绝对卧床并胸腰椎制动；

2. 予以镇痛、补液等对症支持治疗；

3. 请骨科会诊，评估 T8 椎体压缩骨折处理方案（支具固定/住院）；

4. 告知患者及家属警惕腹痛加重、下肢麻木无力等警示症状，必要时立即返院；

5. 建议按会诊意见进一步治疗并复诊。

医师签名：

何贵华

深圳市第二人民医院  
门诊病历专用章

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# Outpatient Medical Record

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|-----------------------------------------|---------------------------------------------|
| <b>Department:</b> Emergency Department | <b>Outpatient No.:</b> 24152016             |
| <b>Name:</b> Hau-sing So                | <b>Contact:</b> 15982372342                 |
| <b>Gender:</b> Male                     | <b>Age:</b> 26 <b>Marital Status:</b> Other |

## 2026-02-27 16:34 — Initial Visit Record

**Chief Complaint:** Chest and back pain with upper abdominal discomfort for 2 hours following traffic accident injury.

**Present Illness:** No significant additional history.

**Past Medical History:** No significant history.

**Allergy History:** None.

**Physical Examination:**

- 1. T8 vertebral compression fracture with approximately 1/4 vertebral body height reduction; no significant spinal canal narrowing at the corresponding level.
- 2. No obvious traumatic changes identified in the liver, gallbladder, pancreas, spleen, bilateral kidneys, or visualized bowel segments.

**Laboratory / Auxiliary Examinations:** Upper and lower abdominal CT plain scan; thoracic spine CT plain scan.

**Preliminary Diagnosis:**

- 1. T8 thoracic vertebral compression fracture
- 2. Closed abdominal injury — to be excluded (no definite solid organ injury on CT)

**Management Plan:**

- 1. Emergency observation; continuous vital sign monitoring; strict bed rest with thoracic-lumbar immobilization ordered.
- 2. Analgesic, fluid resuscitation, and other symptomatic supportive treatment administered.
- 3. Orthopedic consultation requested to evaluate management of T8 vertebral compression fracture (brace fixation vs. hospitalization).
- 4. Patient and family advised to watch for warning signs including worsening abdominal pain and lower limb numbness/weakness; instructed to return immediately if such symptoms develop.
- 5. Further treatment and follow-up to be guided by orthopedic consultation recommendations.

Physician Signature: \_\_\_\_\_

**Reminder: Please bring this medical record at every follow-up visit. Thank you!**





## 门诊病历

科室：急诊科

姓名：王梦竹

性别：女

门诊号：24152019

联系方式：13880065503

年龄：36 岁

婚姻状况：已婚

### 2026-02-27 16:58 初诊记录

**主 诉：**车祸伤后右腕疼痛伴肿胀、旋转受限 4 小时

**现 病 史：**患者为同车交通事故伤，伤后神志清楚，无昏迷及呕吐，右腕着力后出现持续性疼痛和肿胀，旋前旋后活动受限，急诊就诊。

**既往史：**无

**过敏史：**无

**体格检查：**1. 右腕较对侧肿胀，远端桡尺关节及 TFCC 体表投影区压痛 (+)，被动旋前旋后试验 (+)。

2. 右手末梢血运可，指端感觉存在，腕关节主动活动受限。

**检验检查：**1. MRI 示右桡骨远端骨髓水肿，TFCC 损伤征象。

2. CT 示右桡骨远端未见明显骨折，已行右腕关节平扫及三维重建（螺旋扫描）。

**初步诊断：**1. 右腕关节痛（创伤后） 2. TFCC 损伤（右） 3. 右桡骨远端骨挫伤

**处 理：**1. 检查：已完善右腕关节 CT 平扫及三维重建（螺旋扫描）并结合 MRI 综合评估。

2. 治疗：行石膏固定术（中），采用过肘半旋后位石膏或支具固定 4 周，继以短臂固定 2 周。

3. 予以镇痛及消肿对症处理，抬高患肢，避免患侧负重及扭转动作。

4. 建议固定期后逐步进行腕关节功能康复训练

医师签名：\_\_\_\_\_



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# Outpatient Medical Record

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|-----------------------------------------|-----------------------------------------------|
| <b>Department:</b> Emergency Department | <b>Outpatient No.:</b> 24152019               |
| <b>Name:</b> Mengzhu Wang               | <b>Contact:</b> 13880065503                   |
| <b>Gender:</b> Female                   | <b>Age:</b> 36 <b>Marital Status:</b> Married |

## 2026-02-27 16:58 — Initial Visit Record

**Chief Complaint:** Right wrist pain with swelling and limited forearm rotation for 4 hours following traffic accident injury.

**Present Illness:** The patient was injured in the same traffic accident. Consciousness remained clear throughout; no loss of consciousness or vomiting occurred. The right wrist sustained a direct impact, resulting in persistent pain, swelling, and restricted pronation and supination. The patient presented to the emergency department for evaluation.

**Past Medical History:** None.

**Allergy History:** None.

**Physical Examination:**

- 1. Right wrist more swollen than the contralateral side; tenderness over the distal radioulnar joint and the surface projection of the TFCC (+); passive pronation/supination stress test (+).
- 2. Right hand distal circulation intact; digital sensation preserved; active wrist movement restricted.

**Laboratory / Auxiliary Examinations:**

- 1. MRI: Bone marrow edema of the right distal radius with imaging signs consistent with TFCC injury.
- 2. CT: No obvious fracture of the right distal radius; right wrist joint plain scan and three-dimensional reconstruction (spiral scan) completed.

**Preliminary Diagnosis:**

- 1. Right wrist joint pain (post-traumatic)
- 2. TFCC injury (right)
- 3. Right distal radius bone contusion

**Management Plan:**

- 1. **Imaging:** Right wrist CT plain scan and 3D reconstruction combined with MRI for comprehensive assessment completed.
- 2. **Immobilization:** Above-elbow cast in semi-supinated position (or equivalent splint) applied for **4 weeks**, followed by short-arm fixation for an additional **2 weeks**.
- 3. Analgesic and anti-swelling symptomatic treatment administered; affected limb elevated; weight-bearing and twisting of the affected side to be avoided.

4. Gradual wrist functional rehabilitation training recommended after the fixation period.

Physician Signature: \_\_\_\_\_

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**Reminder: Please bring this medical record at every follow-up visit. Thank you!**



## 门诊病历

科室：急诊科

姓名：柳黛鲜

性别：女

门诊号：24152018

联系方式：18652973418

年龄：24 岁

婚姻状况：其他

### 2026-02-27 16:49 初诊记录

**主 诉：**车祸伤后腰背部疼痛伴翻身困难 2 小时

**现 病 史：**患者为同车交通事故伤，伤后无意识丧失及抽搐，主要表现为腰背部持续性疼痛，体位改变时明显，伴轻度腹部不适，急诊就诊。

**既往史：**无

**过敏史：**无

**体格检查：**1. 腰 1 旁开压痛明显，叩击痛 (+)，腰部活动受限。

2. 双下肢感觉运动基本正常，肌力 5 级，足背动脉可触及，生命体征平稳。

**检验检查：**1. 胸腰椎 CT 示 L1 左侧横突线样骨折影，无明显移位。

2. 上腹部 + 下腹部 CT 平扫未见腹腔游离气体或明显实质脏器损伤征象。

**初步诊断：**1. 腰椎 L1 横突骨折（非移位） 2. 腰背部软组织挫伤 3. 腹部闭合性损伤待排除

**处 理：**1. 急诊观察 24 小时，严密监测疼痛变化及神经系统体征；

2. 给予镇痛、补液及抑酸护胃等对症支持治疗；

3. 建议佩戴腰围固定 4-6 周，避免久坐、久站及负重；

4. 嘱出现腹痛加重、下肢麻木无力、大小便异常时立即返院；

5. 建议病休约 12 周（约 3 个月），按 2 周、6 周、10 周节点复诊并逐步康复。

医师签名：刘畅



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# Outpatient Medical Record

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|-----------------------------------------|---------------------------------------------|
| <b>Department:</b> Emergency Department | <b>Outpatient No.:</b> 24152018             |
| <b>Name:</b> Daixian Liu                | <b>Contact:</b> 18652973418                 |
| <b>Gender:</b> Female                   | <b>Age:</b> 24 <b>Marital Status:</b> Other |

## 2026-02-27 16:49 — Initial Visit Record

**Chief Complaint:** Low back pain with difficulty turning over for 2 hours following traffic accident injury.

**Present Illness:** The patient was injured in the same traffic accident. No loss of consciousness or convulsions occurred. The primary presentation is continuous low back pain significantly aggravated with changes in body position, accompanied by mild abdominal discomfort. The patient presented to the emergency department for evaluation.

**Past Medical History:** None.

**Allergy History:** None.

**Physical Examination:**

- 1. Marked tenderness at the L1 paravertebral region; percussion tenderness (+); restricted lumbar mobility.
- 2. Sensation and motor function of bilateral lower limbs essentially normal; muscle strength grade 5; dorsal pedis artery palpable; vital signs stable.

**Laboratory / Auxiliary Examinations:**

- 1. Thoracolumbar CT: Linear fracture shadow at the left transverse process of L1 without significant displacement.
- 2. Upper and lower abdominal CT: No free intraperitoneal gas or obvious solid organ injury signs identified.

**Preliminary Diagnosis:**

- 1. Lumbar L1 transverse process fracture (non-displaced)
- 2. Lumbar soft tissue contusion
- 3. Closed abdominal injury — to be excluded

**Management Plan:**

- 1. 24-hour emergency observation; close monitoring of pain progression and neurological signs.
- 2. Analgesic, fluid resuscitation, and gastroprotective symptomatic supportive treatment administered.
- 3. Lumbar brace fixation recommended for **4–6 weeks**; avoid prolonged sitting, prolonged standing, and heavy lifting.



4. Patient instructed to return immediately if worsening abdominal pain, lower limb numbness/weakness, or bladder/bowel dysfunction develops.
5. Medical leave of approximately **12 weeks** (approximately 3 months) recommended; follow-up at 2-week, 6-week, and 10-week intervals with gradual rehabilitation.

Physician Signature: \_\_\_\_\_

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**Reminder: Please bring this medical record at every follow-up visit. Thank you!**



## 门诊病历

科室：急诊科

姓名：冯辰彬

性别：男

门诊号：24152017

联系方式：13867294531

年龄：42 岁

婚姻状况：已婚

### 2026-02-27 16:41 初诊记录

**主 诉：**车祸伤后颈部疼痛

**现 病 史：**患者于 3 小时前乘坐小型轿车发生追尾碰撞，伤后出现颈部疼痛及僵硬，转颈时明显加重；神志清楚，无昏迷、恶心呕吐及肢体无力，后由家属陪同来急诊就诊。

**既往史：**既往有颈椎病史，间断颈肩部酸痛。

**过敏史：**无

**体格检查：**1. 颈后正中及双侧旁开压痛 (+)，颈椎活动受限，屈伸及旋转受限明显。

2. 双上肢肌力 5 级，感觉未见明显减退，病理征未引出，生命体征平稳。

**检验检查：**1. 颈椎 CT 未见明确骨折及脱位，颈椎生理曲度变直。

2. 颈椎 MRI 示 C5/6、C6/7 椎间盘轻度突出，颈后软组织水肿信号。

**初步诊断：**1. 颈部软组织损伤（鞭打样损伤） 2. 颈椎病（C5/6、C6/7 椎间盘轻度突出）

**处 理：**1. 急诊处理后病情平稳，给予镇痛、肌松及神经营养等对症支持治疗；

2. 建议短期佩戴颈托保护，避免久低头、突然转颈及负重劳动；

3. 指导热敷及颈肩部轻柔康复训练，症状缓解后逐步恢复活动；

4. 建议 2 周、6 周门诊复诊，必要时复查颈椎 MRI 评估恢复情况；

5. 建议病休约 12 周，并持续进行颈椎功能康复训练。

医师签名：

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# Outpatient Medical Record

|                                         |                                               |
|-----------------------------------------|-----------------------------------------------|
| <b>Department:</b> Emergency Department | <b>Outpatient No.:</b> 24152017               |
| <b>Name:</b> Chenbin Feng               | <b>Contact:</b> 13867294531                   |
| <b>Gender:</b> Male                     | <b>Age:</b> 42 <b>Marital Status:</b> Married |

## 2026-02-27 16:41 — Initial Visit Record

**Chief Complaint:** Neck pain following traffic accident injury.

**Present Illness:** The patient was involved in a rear-end collision while riding in a passenger vehicle approximately 3 hours ago. Following the injury, he developed neck pain and stiffness that was significantly aggravated with neck rotation. Consciousness remained clear throughout; no loss of consciousness, nausea, vomiting, or limb weakness was reported. The patient was brought to the emergency department accompanied by family members.

**Past Medical History:** History of cervical spondylosis with intermittent neck and shoulder soreness.

**Allergy History:** None.

**Physical Examination:**

- 1. Tenderness at the posterior cervical midline and bilateral paravertebral regions (+); restricted cervical range of motion with marked limitation in flexion/extension and rotation.
- 2. Bilateral upper limb muscle strength grade 5; no significant sensory deficit; no pathological reflexes elicited; vital signs stable.

**Laboratory / Auxiliary Examinations:**

- 1. Cervical CT: No definite fracture or dislocation; straightening of the cervical physiological curvature.
- 2. Cervical MRI: Mild disc herniation at C5/6 and C6/7; posterior cervical soft tissue edema signal.

**Preliminary Diagnosis:**

- 1. Cervical soft tissue injury (whiplash injury)
- 2. Cervical spondylosis (mild disc herniation at C5/6 and C6/7)

**Management Plan:**

- 1. Condition stabilized after emergency treatment; analgesic, muscle relaxant, and neurotrophic symptomatic supportive therapy administered.
- 2. Short-term cervical collar protection recommended; advised to avoid prolonged forward head posture, sudden neck rotation, and heavy lifting.
- 3. Instructed on hot compress application and gentle cervical rehabilitation exercises; gradual resumption of activity as symptoms improve.

4. Follow-up visits recommended at 2 weeks and 6 weeks; repeat cervical MRI if necessary to assess recovery.
5. Medical leave of approximately **12 weeks** recommended, with continuous cervical functional rehabilitation training.

Physician Signature: \_\_\_\_\_

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**Reminder: Please bring this medical record at every follow-up visit. Thank you!**